[redacted]

FAX

Date: [redacted]

To: Recup Care

Subject: New Referral ED patient

Fax: [redacted]

Pages:

Comments: Please call me @ [redacted]

w/ any questions/updates thanks!

[redacted]

The information contained in this fax transmission is the private and confidential property of the patient and the sender. This information is intended solely for the recipient indicated above. If you are not the intended or their authorized designee, you are hereby notified that any review, disclosure, coping, distribution, or the taking of any action in reliance on the contents of this transmission is strictly prohibited and [redacted] result in legal liability on your part. If you have received this transmission in error, please notify us immediately at the number above and arrange for return of this transmission to us.

[redacted]

[redacted]

Admit Date: [redacted]
Inp Admit Date/Time: N/A N/A
Hospital Account: [redacted]
MRN:
Patient:
Contact Serial #:
Private Encounter?

ENCOUNTER

Patient Class: **Emergency**
Hospital Service: **Emergency Medicine**
Admitting Provider:
Attending Provider: [redacted]
Admit Source: **Non-healthcare facility ***
Advanced Directives: **No-Information Given**

Unit: [redacted]
Room/Bed: [redacted]
Referring Physician:
Adm Diagnosis:
Clinical Impression:
Admit Type: **Emergency**

PATIENT

Name: [redacted]
Address:

Primary Care [redacted]

DOB: [redacted]
Legal Sex: **Female**
Gender Identity:
Patients Primary Phone: [redacted]
PCP Phone: [redacted]

EMERGENCY CONTACT

Contact Name	Legal Guardian?	Relationship to Patient	Home Phone	Primary Phone
1. *No Contact Specified*				
2. *No Contact Specified*				

GUARANTOR

Guarantor: [redacted]
Address:

Relation to Patient:
Guarantor ID:

DOB: [redacted]
Sex:
Home Phone: [redacted]

GUARANTOR EMPLOYER

Employer:
Status: **RETIRED**

COVERAGE

PRIMARY INSURANCE

Insurance Company: **MEDICARE**
Group Number:
Subscriber Name: [redacted]
Pat. Rel. to Subscriber:
Phone Number:

Plan: **MEDICARE PART A AND B**
Insurance Type: **Indemnity [1]**
Subscriber DOB: [redacted]
Subscriber ID:
HIC #:
Billing Flag:

SECONDARY INSURANCE

Insurance Company: **CAL OPTIMA MEDICAID HMO**
Group Number:
Subscriber Name: [redacted]
Pat. Rel. to Subscriber: **Self**
Phone Number:

Plan: **CAL OPTIMA DIRECT MDCD HMO**
Insurance Type: **Indemnity [1]**
Subscriber DOB: [redacted]
Subscriber ID:
HIC #:

[redacted]

[redacted]

LA/OC REFERRAL FORM

To refer a patient, please complete the entire form including supporting documents and **FAX** to [redacted] or **email** to [redacted] For all referral questions, please contact IF Referral Coordinator at [redacted]

Referring Hospital: [redacted] **Dept/Floor** [redacted]
SW/CM/RN [redacted] **Phone/pager#:** [redacted]
E-mail: [redacted] **Nursing station#:** [redacted]
Authorized by: _____ **phone#:** _____

Patient Name [redacted] **MRN** [redacted] **Date of Birth:** [redacted]

Gender: ☐ Male ☒ Female ☐ Transgender **Ethnicity:** Caucasian **Social Security# (last 4)** [redacted]

☒ **ED Visit** ☐ **Hospital Admit** **Da** [redacted] **Expected Discharge Date:** [redacted]

Insurance Type: Medi - Medi **English Speaking?** ☒ Yes ☐ No ***Primary Language Spoken:** _____

PLEASE VERIFY: ☒ **Currently homeless** **Check if:** ☐ **Homeless for more than 1 year continuously or 4 episodes in last 3 years**

CHIEF COMPLAINT/ADMITTING DIAGNOSIS:
Right leg pain, cellulitis

SUBSTANCE ABUSE? ☒ None ☐ Alcohol ☐ Cocaine ☐ Heroin ☐ Meth ☐ Other: _____
Last Date Used: ____/____/_____ Current withdrawals: ☒ No ☐ Yes *Please explain_____

ANY WOUNDS? ☐ Yes ☒ No #/Loc/Size/Stage:_____

Independent with wound care? ☐ Yes ☐ No If no, will Home Health be ordered? ☐ Yes ☐ no

PPD/TB test or Chest XRAY performed? ☒ yes ☐ no Dat[redacted] Results: Neg

ANY LIMITATIONS, BEHAVIORAL CHALLENGES OR MENTAL HEALTH ISSUES? ☐ None ☐ Auditory/Visual Hallucinations

☒ Mental Health DX IF yes, **what type?:** Bipolar, Schizophrenia ☐ Non-compliant ☐ forgetful

☐ Cognitive Impairment ☐ Reg. Sex offender ☒ other, please explain NO behavioral issues identified.

Requires O2? ☐ Yes ☒ No Self Administer All Meds? ☒ Yes ☐ No *Please explain:_____
Continent of Bowel & Bladder? ☒ Yes ☐ No Colostomy/Ileostomy ☐ Yes ☒ No Foley Catheter ☐ Yes ☒ No
Independent w/all ADL's? ☒ Yes ☐ No *Please explain: Has seated walker
Diabetic? ☒ No If ☐ Yes, then: ☐ Insulin ☐ Oral Meds Communicable Diseases?:_____
Anticoagulants?: ☒ No IF ☐ yes, Requires INR/PT/PTT checks through Home Health or Clinic? _____
Ambulatory? ☒ Yes ☐ No Assistive device? ☐ No ☒ Yes *Please Explain: Seated walker
Does Patient have a car? ☐ Yes ☒ No Spouse/Partner? ☐ Yes ☒ No Service Animal? ☐ Yes ☒ No

Length of Stay authorized in Recuperative Care: 30 **days**

Attached Docs: ☒ **Face Sheet** ☒ **H&P** ☒ **Med List** ☒ **Psych notes** ☒ **Surgical notes** ☒ **PT/OT Eval.** ☐ **SW notes**

IF STAFF ONLY

Date Received:_____ Reviewed by:_____ Approved: ☐ Yes ☐ No Date:_____ Auth#_____
Denied Reason?:_____ Admission Date/time:_____ Exit Date:_____
Extension Requested?: ☐ no ☐ yes If yes, Approved additional days?:_____ by:_____ TOTAL LOS:_____

[redacted]

CalOptima Health

CalAIM Recuperative Care/ Short-Term Post Hospitalization Referral/Authorization Request

Member Name: [redacted] CIN: [redacted]

Step 1: Complete all information below.

Referral Information:

Referral Date: [redacted] Referred by: [redacted]

Agency or Relationship to Member: [redacted] Referring Provider NPI (if applicable): _____

Phone: [redacted] Fax: [redacted] Email: [redacted]

Member Information:

Member Name: [redacted]

Member Date of Birth: [redacted] Primary Care Provider: [redacted]

Phone: [redacted] Email: _____

Fax: _____ Social Worker/Case Manager/RN _____

Member's Preferred Language: English

Is Member Currently in Hospital? ☒ Yes No Social Security (last 4 digits): [redacted]

Step 2. [redacted] the boxes for the Community Supports the member is interested in receiving. The following pages provide additional eligibility information about Community Supports.

Step 3: Please check off the most appropriate Recuperative Care/ Short- Term Post Hospitalization Housing pathway based on the eligibility listed below.

☒	**Recuperative Care Only** **(Up to 90 days)**	Short-term residential care for individuals who no longer require hospitalization, but still need to heal from an injury or illness or mental health condition.	**Select one that applies:** ☒ Homeless or at risk of homelessness ☐ Individuals who are at risk of hospitalization or are post-hospitalization, ☐ Individuals who live alone with no formal supports.

[redacted]

CalOptima Health

CalAIM Recuperative Care/ Short-Term Post Hospitalization Referral/Authorization Request

Member Name: [redacted] CIN: [redacted]

☐	**Short-Term Post Hospitalization Housing Only** **(Up to 6 months)** ***Once in a lifetime benefit**	Assist member with high medical or behavioral health needs with short-term housing after leaving the hospital, recovery facility, recuperative care or other facility.	**Select all that apply:** ☐ Homeless or at risk of homelessness **AND** ☐ Member is exiting Recuperative Care, inpatient hospital, residential substance use disorder treatment facility, residential mental health treatment facility, correctional facility or nursing facility.
☐	**Nursing Facility with plans to transition to Recuperative Care**	Short-term residential care for individuals who no longer require hospitalization, but still need to heal from an injury or illness or mental health condition.	**Select one that applies:** ☐ Homeless or at risk of homelessness ☐ Individuals who are at risk of hospitalization or are post-hospitalization, **OR** ☐ Individuals who live alone with no formal supports.
☐	**Recuperative Care with plans to transition to Short-Term Post-Hospitalization Housing**	Short-term residential care for individuals who no longer require hospitalization, but still need to heal from an injury or illness on mental health condition **Transition to:** Assist member with high medical or behavioral health needs with short-term housing after leaving the hospital, recovery facility, recuperative care or other facility.	**Select all that apply:** ☐ Homeless or at risk of homelessness ☐ Individuals who are at risk of hospitalization or are post-hospitalization, **OR** ☐ Individuals who live alone with no formal support. **AND** ☐ Member is exiting Recuperative Care, inpatient hospital, residential substance use disorder treatment facility, residential mental health treatment facility, correctional facility or nursing facility.

[redacted]

CalOptima Health

CalAIM Recuperative Care/ Short-Term Post Hospitalization Referral/Authorization Request

Member Name: [redacted] CIN: [redacted]

Admitting Diagnosis

Qualifying Recuperative Care/ Short-Term Post Hospitalization Housing diagnosis:
right leg cellulitis

ED Visit / Hospital Admit Date [redacted] Expected Discharge Date: [redacted]

Post-Acute Discharge Instructions

Clients last visit with their PCP: _____ Their Next visit: _____

Are there any specialists that member will need to follow up with?

Specialty: None Provider Name: _____
Phone Number: _____ Scheduled Appt Date: _____
Specialty: _____ Provider Name: _____
Phone Number: _____ Scheduled Appt Date: _____
Specialty: _____ Provider Name: _____
Phone Number: _____ Scheduled Appt Date: _____

Authorized Home Health Provider

Service: ☐ PT ☐ OT ☐ Speech ☐ Wound Care Provider Name: [redacted]
Phone Number: _____ Scheduled Appt Date: _____

Health Information

General:

Gender: ☐ Male ☒ Female ☐ Placement needs related to gender
COVID vaccine? Dose 1: ☐ YES ☐ NO Dose 2: ☐ YES ☐ NO
Booster 1: ☐ YES ☐ NO Booster 2: ☐ YES ☐ NO
TB Test or Chest X-Ray Performed? ☒ YES ☐ NO Date [redacted] Results: ☐ Positive ☒ Negative

Neuro:

Alert and Oriented to: ☒ Person ☒ Place ☒ Time ☒ Situation

Respiratory:

☐ Requires O2 (Explain): None

[redacted]

CalOptima Health

CalAIM Recuperative Care/ Short-Term Post Hospitalization Referral/Authorization Request

Member Name: [redacted] CIN: [redacted]

GI/ GU:

☐ Incontinent of bowel ☐ Incontinent of bladder ☐ Colostomy/Ileostomy ☐ Foley Catheter
Does the client require tube feeding? ☐ YES ☒ NO

Ambulation/ Mobility:

Can the client independently perform ADLs? ☒ YES ☐ NO
Does the Client use DME? ☒ YES ☐ NO
(Explain) Seated walker
Fractures ☐ YES ☒ NO Recent Surgery ☐ YES ☒ NO

Integumentary:

Wound(s) ☐ YES ☒ NO
Location(s)/Size/Stage:
Independent with wound care ☐ YES ☐ NO

Infections:

Communicable Diseases/Isolation describe:
IV Antibiotics: ☐ YES ☒ NO Frequency:

Psycho-Social Information:

☐ Registered Sex Offender
Client has a: ☐ Car ☐ Spouse/Partner ☐ Service Animal ☐ Pets

Substance Use: ☒ None ☐ Alcohol ☐ Cocaine ☐ Heroin ☐ Methamphetamine ☐ Opioid
Other: Last Date Used:

Mental Health DX: No behavorial needs identified
☐ Anxiety ☒ Bipolar ☐ Cognitive Impairment ☐ Depression ☒ Schizophrenia ☐ Trauma-related
☐ Other *Please explain ☐ Current treatment:
Requires assistance with ADL's (Explain): NO

Chronic Disease Management:

☐ Diabetic ☐ Insulin ☒ Oral Meds ☐ Anticoagulants ☐ Requires INR/PT/PTT checks
☐ Requires Assistance with Medication
List medication(s) :
Does member have enough medication to last through the end of the month? ☒ YES ☐ NO
Does the member understand how to obtain refills on their medications? ☒ YES ☐ NO
Does the member have a preferred pharmacy? ☐ YES ☒ NO
If Yes, where:
Does the member understand how to take their medication and why they are taking their medication?
☒ YES ☐ NO

[redacted]

CalOptima Health

CalAIM Recuperative Care/ Short-Term Post Hospitalization Referral/Authorization Request

Member Name: [redacted] CIN: [redacted]

Step 4: Based on the services selected for the member above, please submit this referral form to the most appropriate provider listed below via fax or mail.

Community Supports Provider Contact Information

Name	Phone Number	Fax Number	E-Mail Address
Recuperative Care			
[redacted]			
Recuperative Care and Short-Term Post Hospitalization Housing			
[redacted]	[redacted]		[redacted]

----------STOP----------

Community Support Provider to complete this section

Step 5: Complete section below and return the response to the referrer at the Hospital/ Skilled Nursing Facility. If member belongs t[redacted]se submit these documents directly to [redacted]

Accepting/ Not Accepting:

Was the member accepted? ☐ YES ☐ NO

If member Declined- What is reason member was declined?

_____

_____

[redacted]

[redacted]

[redacted]

[redacted]

Registered Nurse
Specialty: Registered Nurse

ED Notes
Signed

[redacted]

Patient is able to independently manage her own ADLS and ambulate 100ft with the assistance of her walker.

[redacted]

Department

[redacted] (73 y.o. F) [redacted]

ED Diagnosis

None

Results

Procedure	Component	Value	Ref Range	Date/Time
Urinalysis, Reflex Microscopic and/or Cultu[redacted]				[redacted]
Order Status: Sent		Specimen: Urine, Clean Catch		
Influenza A,B,RSV,COVID-19, NAAT [redacted] Normal)				
Order Status: Completed		Specimen: Body Fluid from Nasopharynx		
	Influenza A, Qual, NAAT	Negative	Negative	
	Influenza B, Qual, NAAT	Negative	Negative	
	RSV, Qual, NAAT	Negative	Negative	
	SARS-CoV-2, NAAT (COVID-19)	Negative	Negative	
Procalcitonin [redacted] (Abnormal)				[redacted]
Order Status: Completed		Specimen: Blood		
	Procalcitonin	[redacted] ^	[redacted] ng/mL	

Narrative:

Procalcitonin Clinical Guidelines

[redacted] ng/mL Low risk for progression to severe [redacted] and/or [redacted] shock [redacted] ng/mL [redacted] should be considered. Recommend retest within [redacted] hours. [redacted] ng/mL High risk for progression to [redacted] shock

Procalcitonin levels must always be interpreted in the context of other laboratory findings and clinical assessments.

Lactic Acid Reflex to RPT 2HR If Indicated [redacted] Normal)				[redacted]
Order Status: Completed		Specimen: Blood		
	Lactate	0.8	[redacted] mmol/L	
Comprehensive Metabolic Pan [redacted] (Abnormal)				[redacted]
Order Status: Completed		Specimen: Blood		
	Sodium	140	136 - 145 mmol/L	
	Potassium	[redacted]	[redacted] mmol/L	
	Chloride	105	97 - 107 mmol/L	
	Carbon Dioxide	28	20 - 31 mmol/L	
	Anion Gap	7	[redacted] mmol/L	
	Glucose	101	74 - 106 mg/dL	

[redacted]

Procedure	Component	Value	Ref Range	Date/Time
	BUN	16	9 - 23 mg/dL	
	Creatinine	0.54 ⌄	[redacted] - [redacted] mg/dL	
	Calcium	9.8	[redacted] mg/dL	
	Albumin	4.4	[redacted] - [redacted] g/dL	
	Bilirubin Total	0.4	[redacted] - [redacted] mg/dL	
	Total Protein	[redacted]	[redacted] - [redacted] g/dL	
	AST	14	<34 U/L	
	ALT	10	10 - 49 U/L	
	Alkaline Phosphatase	71	46 - 116 U/L	
	Globulin	[redacted]	g/dL	
	Albumin/Globulin Ratio	[redacted]		
	BUN/Creatinine Ratio	[redacted]		
	eGFR	97	>=60 mL/min/1.73m2	
CBC with Di[redacted] (Abnormal)				[redacted]
Order Status: Completed		Specimen: Blood		
	White Blood Cells	7.46	[redacted] - [redacted] K/uL	
	Red Blood Cells	[redacted]	[redacted] - [redacted] M/uL	
	Hemoglobin	[redacted]	[redacted] g/dL	
	Hematocrit	[redacted]	[redacted] - [redacted] %	
	MCV	[redacted]	[redacted] - [redacted] fL	
	MCH	[redacted]	[redacted] - [redacted] pg	
	MCHC	[redacted]	[redacted] - [redacted] g/dL	
	RDW-CV	[redacted]	[redacted]	
	RDW-SD	44.4	[redacted] - [redacted] fL	
	Platelet Count	362	160 - 370 K/uL	
	MPV	8.8 ⌄	[redacted] fL	
	% Neutrophils	[redacted]	[redacted] - [redacted]	
	% Lymphocytes	[redacted]	[redacted] - [redacted]	
	% Monocytes	[redacted]	[redacted]	
	% Eosinophils	0.9	[redacted] - [redacted]	
	% Basophils	[redacted]	[redacted]	
	% Immature Granulocytes	[redacted]	[redacted]	
	Absolute Neutrophils	[redacted]	[redacted] - [redacted] K/uL	
	Absolute Lymphocytes	[redacted]	[redacted] - [redacted] K/uL	
	Absolute Monocytes	[redacted]	[redacted] - [redacted] K/uL	
	Absolute Eosinophils	[redacted]	[redacted] - [redacted] K/uL	
	Absolute Basophils	[redacted]	[redacted] - [redacted] K/uL	
	Absolute Immature Granulocytes	[redacted]	[redacted] - [redacted] K/uL	
	% nRBC	[redacted]	[redacted] - [redacted] per 100 WBCs	
	Absolute nRBC	[redacted]	[redacted] - [redacted] K/uL	
Culture, Bloo[redacted]				[redacted]
Order Status: Sent		Specimen: Peripheral Blood		
Culture, Bloo[redacted]				
Order Status: Sent		Specimen: Peripheral Blood		

Imaging Results

[redacted]

XR Foot Right 3 + Vw (In process)

VAS Lower Extremity Venous Bilateral (In process)

XR Chest AP Portable (Final result)

[redacted]

Final result[redacted]

Impression:

IMPRESSION:

No acute thoracic abnormality.

Electronically signed by[redacted]

Narrative:

EXAMINATION: XR CHEST AP PORTABL[redacted]

CLINICAL HISTORY: 73 years of age, Female,[redacted]

COMPARISON: Chest x-ray on[redacted]

FINDINGS:

Lungs are clear. No pleural effusion or pneumothorax. Aortic knob calcifications. Normal cardiac silhouette size. Upper abdomen is unremarkable. No acute osseous findings.

ECG Results

ECG 12 lead (Preliminary result)

Collection Time	Result Time	INTERPRETATION TEXT
[redacted]		Not Confirmed

Medication Administration - last 24 hours from[redacted]

Date/Time	Order	Dose	Route	Action	Action by
[redacted]	cefepime (MAXIPIME) 2 g in sterile water 20 mL syringe	2 g	Intravenous	Given	[redacted]
	lactated ringers (LR) bolus 1,000 mL	1,000 mL	Intravenous	New Bag	
	vancomycin in saline IVPB 2 g	2 g	Intravenous	New Bag	

Home Medications

	Taking?	Last Dose	Start Date	End Date	Provider
acetaminophen (TYLENOL) 500 mg tablet		--	--	--	Historical Provider, MD
acetaminophen-codeine (TYLENOL #3) [redacted] mg per tablet		--	[redacted]	--	Home Care Services
apixaban (ELIQUIS) 5 mg tablet Take 1 tablet by mouth 2 times daily.		--		--	[redacted]
cetirizine (ZYRTEC) 10 mg tablet		--		--	Historical Provider, MD

[redacted]

	Taking?	Last Dose	Start Date	End Date	Provider
cyclobenzaprine (FLEXERIL) 5 MG tablet		--	--	--	Historical Provider, MD
hydrocortisone (PREPARATION H) 1% rectal cream Place rectally 4 times daily as needed for Hemorrhoidal Pain.		--	[redacted]	--	[redacted]
Levalbuterol Tartrate (XOPENEX HFA INHAL)		--		--	
losartan (COZAAR) 50 mg tablet Take 1 tablet by mouth Daily.		--		--	
polyethylene glycol (MIRALAX) 17 g packet Take 1 diluted packet by mouth Daily as needed for Constipation (IF LACTULOSE INEFFECTIVE).		--		--	

Allergies (Review Complete on [redacted]

Agent	Severity	Comments
Menthol	**High**	**Anything with menthol triggers asthma**
Sulfa Antibiotics	**High**	
Erythromycin		**Facial swelling**
Adhesive & Tape	**Low**	
Latex	**Low**	

ED Vitals (Past 4H) from [redacted]

Date/Time	Temp	Pulse	Resp	BP	SpO2	O2 Device	Who
[redacted]	[redacted] °F)	72	16	178/79	98 %	room air	[redacted]
	--	--	--	--	--	room air	

ED A/B/C/D Assess

Date/Time	Respiratory WDL	[READ ONLY] Airway WDL	[READ ONLY] Respiratory WDL	Cardiac WDL	[READ ONLY] Cardiac WDL	(READ ONLY) Cognitive/Neuro/Behavioral WDL	Who
[redacted]	WDL	--	--	WDL	--	--	[redacted]

Selected ED Notes

No notes of this type exist for this encounter.

[redacted]

[redacted]

[redacted] ED Provider Notes [redacted]

Emergency Medicine Shared

[redacted]

[redacted]

[redacted]

Mode of Arrival: BLS Ambulance
Accompanied by: EMS
[redacted]

Nursing Triage Note

ED Triage [redacted]

Pt biba from grocery store parking lot. Pt has R leg pain, diagnosed with cellulitis about a month ago. Pt states she has been homeless since Friday. Pt is able to bear some weight.

CHIEF COMPLAINT

Chief Complaint
Patient presents with
- Leg Pain

HISTORY OF PRESENT ILLNESS

[redacted]

[redacted] 73 y.o. female, with past medical history of homeslessness for 5 days, anemia, asthma, atrial fibrillation, bipolar 1 disorder, schizophrenia, and sleep apnea, who presents to the emergency department with a chief complaint of right leg pain for about 1 month with associated redness and swelling from her shin to her toes. Patient reports she was diagnosed with cellulitis 8 days ago and has been on a course of Doxycycline for 8 days. She states she had some improvement with the treatment but has felt herself become "sicker" in the last 2 days. She endorses an infection of her leg 3 months ago and was placed on IV antibiotics. Patient reports she was on Eliquis for 6 months for clotting and was taken off it recently when her blood clot cleared. Additionally, she states she has difficulty walking due to back pain. Denies redness of her left leg.

REVIEW OF SYSTEMS

As per HPI for details.

PAST HISTORY

PAST MEDICAL HISTORY

Past Medical History
Past Medical History:

Diagnosis	Date
• Acid reflux disease	
• Anemia	
• Arthritis	
• Asthma (HCC)	
• Atrial fibrillation (HCC)	
• Back injury *from MVA*	
• Back pain	
• Barrett esophagus	
• Bipolar 1 disorder (HCC)	
• Cataract *WITH BIL CATARACT SURGERY*	
• Claustrophobia	
• Paroxysmal atrial fibrillation (HCC)	
• Schizophrenia (HCC)	
• Sleep apnea *USES CPAP MACHINE*	

There is no immunization history on file for this patient.

SURGICAL HISTORY

Past Surgical History
Past Surgical History:

Procedure	Laterality	Date
• ABLATION OF DYSRHYTHMIC FOCUS *Procedure: CV EP ABLATION A-FIB;* [redacted]	Right	[redacted]
• CATARACT REMOVAL	Bilateral	
• CERVICAL POLYP REMOVAL		
• COLONOSCOPY		
• COLONOSCOPY *Procedure: COLONOSCOPY; Laterality: N/A;* [redacted]	N/A	
• DEBRIDEMENT *Procedure: IRRIGATION AND DEBRIDEMENT OF RIGHT THIGH HEMATOMA;* [redacted]	Right	
• TRANSESOPHAGEAL ECHOCARDIOGRAM *Procedure: ECHO - TRANSESEOPHAGEAL / CARDIOVERSION;* [redacted]	N/A	
• UPPER GASTROINTESTINAL ENDOSCOPY *Procedure: EGD; Laterality: N/A;* [redacted]	N/A	

FAMILY HISTORY

Family History
History reviewed. No pertinent family history.
No other family history pertinent to this visit.

SOCIAL HISTORY

Patient lives a[redacted]
Social History
Social History
Tobacco Use

[redacted]

- Smoking status: Former
  - Current packs/day: [redacted]
  - Types: Cigarettes
  - Start date: 1972
  - Quit date: 2022
  - Years since quitting: [redacted]
- Smokeless tobacco: Never

Substance Use Topics

- Alcohol use: Not Currently
- Drug use: Not Currently

MEDICATIONS

PTA Meds

PTA Home Medications

Medication	Sig
acetaminophen (TYLENOL) 500 mg tablet	Take 1 tablet by mouth Twice daily as needed for Pain.
acetaminophen-codeine (TYLENOL #3) [redacted] mg per tablet	Take 1 tablet by mouth every 4 hours as needed for Pain.
apixaban (ELIQUIS) 5 mg tablet	Take 1 tablet by mouth 2 times daily.
cetirizine (ZYRTEC) 10 mg tablet	Take 1 tablet by mouth nightly.
cyclobenzaprine (FLEXERIL) 5 MG tablet	Take 3 tablets by mouth nightly.
hydrocortisone (PREPARATION H) 1% rectal cream	Place rectally 4 times daily as needed for Hemorrhoidal Pain.
Levalbuterol Tartrate (XOPENEX HFA INHAL)	Inhale 2 puffs into the lungs EVERY 4 TO 6 HOURS AS NEEDED (Exacerbation of asthma).
losartan (COZAAR) 50 mg tablet	Take 1 tablet by mouth Daily.
polyethylene glycol (MIRALAX) 17 g packet	Take 1 diluted packet by mouth Daily as needed for Constipation (IF LACTULOSE INEFFECTIVE).

ALLERGIES

Menthol, Sulfa antibiotics, Erythromycin, Adhesive & tape, and Latex

PHYSICAL EXAM

VITAL SIGNS: (first vital signs):Temp: 36.6 °C [redacted] °F) Heart Rate: 72 Resp: 16 SpO2: 98 % BP: 178/79

Encounter Vitals
BP: 178/79
Heart Rate: 72
Resp: 16
Temp: [redacted] °C [redacted] °F)
Temp Source: Oral
SpO2: 98 %

Constitutional: Well developed, well nourished, in no acute distress.
HEENT: Normocephalic, atraumatic, bilateral external ears normal, nose normal. Conjunctiva normal, no discharge.
Neck: Normal range of motion, supple.
Respiratory: Normal breath sounds, no respiratory distress, no wheezing, no accessory muscle use.
Cardiovascular: Well perfused, normal rhythm, no murmurs. DP/TP 2+ BLE
ABD: Soft, no tenderness, no guarding, rigidity or rebound.
Musculoskeletal: No edema, no tenderness, no cyanosis. Good range of motion in all major joints. No major deformities noted.

[redacted]

Skin: Warm, dry. Erythema to the right anterior shin that is blanchable. Slightly hot to touch. No crepitus.
Neurologic: Alert & oriented x4. Moves all 4 extremities well; normal speech. Normal motor function, normal sensory function, no focal deficits noted.
Psychiatric: Affect normal, judgment normal, mood normal

[redacted]

DATA

LABS

Results

Procedure	Component	Value	Ref Range	Date/Time
Influenza A,B,RSV,COVID-19, [redacted] (Normal) Order Status: Completed		Specimen: Body Fluid from Nasopharynx		[redacted]
	Influenza A, Qual, NAAT	Negative	Negative	
	Influenza B, Qual, NAAT	Negative	Negative	
	RSV, Qual, NAAT	Negative	Negative	
	SARS-CoV-2, NAAT (COVID-19)	Negative	Negative	
[redacted] **in** [redacted] **(Abnormal)** Order Status: Completed	**Procalcitonin**	Specimen: Blood 0.10 ^	[redacted] ng/mL	[redacted]

Narrative:
Procalcitonin Clinical Guidelines

[redacted] ng/mL Low risk for progression to severe [redacted] and/or [redacted] shock [redacted] ng/mL [redacted] should be considered. Recommend retest within [redacted] hours. [redacted] ng/mL High risk for progression to [redacted] shock

[redacted]

Procedure	Component	Value	Ref Range	Date/Time

Procalcitonin levels must always be interpreted in the context of other laboratory findings and clinical assessments.

Lactic Acid Reflex to RPT 2HR If Indicate[redacted] [redacted]
(Normal)
Order Status: Completed Specimen: Blood

Lactate	[redacted]	[redacted] mmol/L

Comprehensive Metabolic [redacted] (Abnormal) [redacted]
Order Status: Completed Specimen: Blood

Sodium	140	136 - 145 mmol/L
Potassium	[redacted]	[redacted] mmol/L
Chloride	105	97 - 107 mmol/L
Carbon Dioxide	28	20 - 31 mmol/L
Anion Gap	7	5 - 13 mmol/L
Glucose	101	74 - 106 mg/dL
BUN	16	9 - 23 mg/dL
Creatinine	0.54 ˅	[redacted] mg/dL
Calcium	[redacted]	[redacted] mg/dL
Albumin	4.4	[redacted] g/dL
Bilirubin Total	0.4	[redacted] mg/dL
Total Protein	8.0	[redacted] g/dL
AST	14	<34 U/L
ALT	10	[redacted] U/L
Alkaline Phosphatase	71	46 - 116 U/L
Globulin	[redacted]	g/dL
Albumin/Globulin Ratio	1.2	
BUN/Creatinine Ratio	[redacted]	
eGFR	97	>=60 mL/min/1.73m2

CBC with Differentia[redacted] (Abnormal) [redacted]
Order Status: Completed Specimen: Blood

White Blood Cells	7.46	[redacted] K/uL
Red Blood Cells	4.88	[redacted] M/uL
Hemoglobin	[redacted]	[redacted] g/dL
Hematocrit	41.2	[redacted] %
MCV	[redacted]	[redacted] fL
MCH	[redacted]	[redacted] pg
MCHC	[redacted]	[redacted] g/dL
RDW-CV	[redacted]	[redacted]
RDW-SD	[redacted]	[redacted] fL
Platelet Count	362	160 - 370 K/uL
MPV	8.8 ˅	[redacted] fL
% Neutrophils	[redacted]	[redacted]
% Lymphocytes	[redacted]	[redacted]
% Monocytes	[redacted]	4.7 - 12.5 %
% Eosinophils	[redacted]	[redacted]
% Basophils	0.1	[redacted]
% Immature	0.4	[redacted]

[redacted]

Procedure	Component	Value	Ref Range	Date/Time
	Granulocytes Absolute	[redacted]	[redacted] K/uL	
	Neutrophils Absolute	[redacted]	[redacted] K/uL	
	Lymphocytes Absolute	[redacted]	[redacted] K/uL	
	Monocytes Absolute	0.07	[redacted] K/uL	
	Eosinophils Absolute	[redacted]	[redacted] K/uL	
	Basophils Absolute	0.03	[redacted] K/uL	
	Immature Granulocytes % nRBC	0.0	[redacted] per 100 WBCs	
	Absolute nRBC	0.00	[redacted] K/uL	
Culture, Blood [redacted]				
Order Status: Sent		Specimen: Peripheral Blood		[redacted]
Culture, Blood [redacted]				
Order Status: Sent		Specimen: Peripheral Blood		
Urinalysis, Reflex Microscopic and/or Cultur [redacted]				
Order Status: Sent		Specimen: Urine, Clean Catch		

ED COURSE

Patient Vitals for the past 24 hrs:

	BP	Temp	Temp src	Pulse	Resp	SpO2
[redacted]	178/79	36.6 °C (97.9 °F)	Oral	72	16	98 %

Encounter Vitals
BP: 178/79
Heart Rate: 72
Resp: 16
Temp: [redacted] °C [redacted] °F)
Temp Source: Oral
SpO2: 98 %

Pertinent Labs & Imaging studies reviewed.
Quality measures reviewed.

PROCEDURES

PROGRESS NOTES

MEDICAL [redacted] MAKING

[redacted] presented to the Emergency Department for evaluation, and she was triaged to room [redacted] reviewed the nursing notes, and she was evaluated by me.

[redacted]

Independent historian (e.g. parents, caregiver, EMS crew, law enforcement):

History obtained from patient.

External notes/Records interpreted by me (e.g. DC summary, outpatient records, EMS reports, outside facilities) WITH SUMMARY:

None

Chronic Medical conditions Interpreted by me and Reviewed Pertaining to this Visit:

Past Medical History
Past Medical History:
Diagnosis Date

- Acid reflux disease
- Anemia
- Arthritis
- Asthma (HCC)
- Atrial fibrillation (HCC)
- Back injury
  from MVA
- Back pain
- Barrett esophagus
- Bipolar 1 disorder (HCC)
- Cataract
  WITH BIL CATARACT SURGERY
- Claustrophobia
- Paroxysmal atrial fibrillation (HCC)
- Schizophrenia (HCC)
- Sleep apnea
  USES CPAP MACHINE

Patient's care impacted by chronic condition of as above.

Medications Requiring Close Monitoring by me for Toxicity or Adverse Effect (Medication and Method of Monitoring[redacted] pulse oximetry, cardiac monitoring):

***

Independent diagnostic interpretation and testing/imaging interpreted by me:

Labs interpreted and reviewed by me: See MDM

EKG reviewed and interpreted by me:
Date[redacted]**
Time: ***
Rate: ***
Rhythm:[redacted]
Interpretation:

Compared to prior: {Blank[redacted]from Prior EKG","Changed from prior EKG","No prior EKG available for comparison"}
Reviewed and interpreted by myself.

XR Chest AP Portable
Final Result
IMPRESSION:

No acute thoracic abnormality.

[redacted]

Electronically signed by [redacted]
10:41 AM PDT

VAS Lower Extremity Venous Bilateral (Results Pending)
XR Foot Right 3 + Vw (Results Pending)

IMAGING interpreted by me: See MDM

Medical Decision-Making/ Differential Diagnosis/Decision or consideration regarding hospitalization and/or surgery:

This is a 73 y.o. female who presents to the ED for ***

[redacted]

{Sepsis Doc [redacted]

I [redacted] am the primary clinician of record.

[redacted] of Management with Additional Providers and Consultants(e.g. hospitalists, specialists, case manager, PET Team, pharmacist):

***

Social Determinants Of Health Affecting Management (example homelessness, food insecurity, unemployment, addiction, underinsured, language issues) WITH ACTION TAKEN (e.g. resource referral, translator utilization, referral to free clinic, arranging for ride):

***

CRITICAL CARE:

***

IMPRESSION

***
No diagnosis found.

DISPOSITION/MEDICATIONS

***
ED Disposition
None

ED Prescriptions

None

Medications

[redacted]

acetaminophen (TYLENOL) tablet 650 mg (has no administration in time range)
vancomycin per pharmacy (has no administration in time range)
lactated ringers (LR) bolus 1,000 mL (has no administration in time range)
cefepime (MAXIPIME) 2 g in sterile water 20 mL syringe (2 g Intravenous Give [redacted]

The patient was provided with a recommendation to follow up with a primary care provider and obtain reassessment of their blood pressure within three months.***

ATTESTATION

I have personally performed the history, exam and services documented above by the scribe. I have reviewed documentation recorded by and described and I find it to be accurate and complete.

By signing my name below [redacted] ical scribe, attest that this documentation has been prepared under the direction and in the presence [redacted]

Record is Dictated with Dragon Dictation System.

Revision History

Date/Time	User	Provider Type	Action
[redacted]	[redacted]	—	Share
		Physician	Share